

CADENCE · JUNO X ANTHROPIC · BUILD THE FUTURE OF HEALTHCARE

Your doctor's appointment doesn't end when you leave the room.

Cadence is an agentic care companion with consistent longitudinal memory. It captures the visit, follows the plan through the weeks after by voice, and walks the patient into the next visit with a brief built from what actually happened.

Clinician-led team

Patient-side, patient-owned

Built in 36 hours

THE PROBLEM

A visit isn't an event. It's the start of an interval.

"Try this for six weeks. Get the blood test. Come back if it gets worse." Then the door closes, and nothing spans what comes next.

The patient forgets

Half of what was said is gone within minutes. The plan lives in a memory, under stress, in jargon.

The doctor forgets

By next week the patient is one of hundreds. What happened in the interval never reaches the clinic.

The next visit starts cold

Ten minutes, and the first eight are spent reconstructing the last visit instead of acting on it.

WHY WE BUILT IT

We are a clinician-led team. We watch patients leave with a plan and come back without one. The uncertainty between visits is where care is lost.

Cadence exists to bring clarity and control back to the patient: what was said, what was agreed, what to do now, and what to bring back. The interval stops being a silence and becomes part of the record.

THE PRODUCT

One loop, every visit

1

Record the visit

Ambient capture with ElevenLabs Scribe. No notes to scribble, nothing to remember on the way out.

2

Understand it instantly

Claude turns the transcript into a plain-language summary and extracts the commitments: the medication, the test, the safety-netting.

3

Follow through, by voice

Autonomous voice-agent check-in calls across the weeks, context-engineered per patient before every call. Thirty seconds, no forms.

4

Walk into the next visit ready

The next-visit brief closes the loop. The appointment starts at minute two, not minute zero.

Next-visit brief

Hypothyroidism · Week 7 · Prepared by the patient

WHAT WE AGREED

Start levothyroxine daily
Blood test at week 6

WHAT I DID

Done Blood test at week 6
Not done Morning dose on empty stomach

WHAT CHANGED

Energy improving since week 4. Early-hours waking began week 5.

The brief is honest, and that is the point

A summary is a memory aid; that has been done. The brief is a synthesis of the whole interval, built from consistent longitudinal memory.

- **Agreed / done / happened / changed.** Four questions every follow-up visit actually needs answered, answered from real interval data.
- **Gaps are stated, never softened.** "Not done" reads as not done. "Never discussed in a check-in" is printed on the sheet. The gap is the signal the doctor needs.
- **It travels.** Patient-owned and portable by construction: to a specialist, an ER, a new city. It works across fragmented care precisely because it is not in the EHR.
- **It compounds.** Stack the loops and the memory becomes a record of a life in the system: what was tried, what worked, what got missed.

Consistent longitudinal memory, agents around it

ElevenLabs Scribe	Ambient capture of the consultation.
ElevenLabs Agents	Autonomous conversational voice agents run the between-visit check-ins, dynamically briefed per patient: pace, hearing, what matters to them, which red-flag questions to ask.
Claude (caretaker LLM)	Summarisation, commitment extraction, interval planning, red-flag triage routing, brief synthesis, and retrieval-grounded ask-your-record Q&A with citations. Every call is schema-validated structured output.
Supabase (Postgres + pgvector)	The memory substrate: the patient-owned longitudinal record of visits, check-ins, briefs, and events.
Knowledge base (CKS / NICE)	Grounds the disease and medication context the interval plan draws on.
Django + Next.js	Every clinical decision is server-side Python: prompts, schemas, triage, safety boundaries. The browser composes no clinical text.

Documents and supports. Never diagnoses or prescribes.

- "The doctor said to reassess X in six weeks; here is how X went" is in scope. "You should change your dose" is not, and the Q&A layer refuses it visibly.
- Red-flag answers in a check-in route to one outcome only: return to the clinic early. Cadence chases follow-through; it never decides care.
- Patient-side is the safe side: self-management, not clinical decision support. The regulatory line is a design feature, not a disclaimer.
- Consent is explicit at every recording, and the record can be erased by the patient at any time.

Run it in two commands

1

```
cp .env.example .env
```

Anthropic, ElevenLabs and Supabase keys.

2

```
docker compose up --build
```

Web on :3000, API on :8000. No accounts, by design: the record is patient-owned.

3

Walk the loop

Get started, record or paste a consultation (fixtures included), take a check-in call, open the brief. The first brief build is a full-interval Claude pass, about a minute, cached afterward.

Repository

github.com/jackysiupuichung/juno_hack_cadenceai

The wedge

The multi-visit chronic patient: many specialists, many "try this and reassess" intervals. For them the loop is not nice-to-have; it is every cycle of their care.

The line

We turn every visit from a cold start into a warm one.